

Saman Vojdani, MD
Orthopedic Surgery

Date of Review: June 24, 2026

Case: Yasmine Watkins v. Adreona Thurman (Case No. 25-A-2515)

Date of Injury: April 28, 2023

Dear Reviewing Party,

I am an orthopedic surgeon currently in practice, actively treating patients currently, and I have been in practice in the State of Georgia since 2020. I am board certified by the American Board of Orthopedic Surgeons. I am fellowship trained in reconstruction of the hip and knee with a practice that services all types of orthopedic complaints. I am the Medical Director for Advanced Center for Joint Surgery – Atlanta, as well as Director of Atlanta for my group, Total Joint Specialists. I formerly held a faculty position at Emory University. Please feel free to contact me for a requested copy of my CV for further details.

The following is a review of the records provided. I am familiar with the standard of care for health care providers in the state of Georgia and aware of the charges for medical care in this region. I reserve the right to amend or supplement my opinions in the future, based on receipt of additional information.

CONFIDENTIAL — ATTORNEY-CLIENT / WORK PRODUCT PRIVILEGE

This opinion is based on the Grady Health System records (MRN 13502505) as provided. Additional records from other providers may alter these opinions.

This expert opinion has been formed through comprehensive review of the Grady Health System electronic medical records for Yasmin Nefreteri Watkins (MRN 13502505; DOB 02/27/1974), spanning encounters from February 2011 through approximately May 2026. The record set comprises 214 supplemental volumes containing in excess of 4,400 pages of clinical documentation, including emergency department records, inpatient admission notes, operative reports, history and physical examinations, nursing notes, radiology reports, laboratory studies, and consultation notes. The matter under review is Yasmine Watkins v. Adreona Thurman, Case No. 25-A-2515, State Court of Cobb County, Georgia. I was retained by defense counsel Jacob Eby of Lynn Leonard & Associates / State Farm Mutual Automobile Insurance Company to opine on

whether the April 28, 2023 motor vehicle accident caused or aggravated the right foot pathology alleged by Plaintiff.

On April 28, 2023, Plaintiff Yasmin Watkins was involved in a motor vehicle accident in Cobb County, Georgia involving Defendant Adreona Thurman. This litigation — Case No. 25-A-2515, filed in the State Court of Cobb County — alleges that injuries sustained in that accident are causally responsible for the right foot pathology and associated surgical intervention that emerged over the course of 2025.

The medical records reviewed from Grady Health System do not contain a contemporaneous emergency department visit, urgent care evaluation, or documented first-report-of-injury following the April 28, 2023 accident. No Grady Health System record in the 4,400-page production documents acute foot pain, foot swelling, foot imaging, or any right foot complaint in temporal proximity to the April 28, 2023 date of loss. The first documented right foot complaint relevant to this action appears in the November of 2023. As chronologically recorded below, this was a separate complaint not related to the Lisfranc injury under review. The first midfoot complaint that is relevant to the Lisfranc injury under review was on a June 24, 2025 visit, where Ms. Watkins explicitly denied any accidents or trauma as a cause of her foot symptoms.

The ICD-10 external cause code “Injury due to impact of moving subject with stationary object” (W22.09XA) and the diagnosis “Lisfranc’s sprain, right, initial encounter” were added to the medical record on July 3, 2025 — the date of surgery — and formally entered into the problem list on July 15, 2025, two weeks post-operatively. These codes were not documented in any encounter following the April 28, 2023 accident.

A. Extensive Pre-Existing Comorbidity Profile

Ms. Watkins presents with a longstanding, extensively documented history of poorly controlled insulin-dependent Type 2 Diabetes Mellitus with multiple end-organ complications. Her HbA1c levels of 9.8% (December 2021) and 9.0% (May 2025) reflect chronic hyperglycemia substantially above therapeutic targets over a multi-year period. The medical record documents severe diabetic polyneuropathy with absent protective sensation bilaterally, proliferative diabetic retinopathy, diabetic gastroparesis, Chronic Kidney Disease Stage 2, Charcot neuroarthropathy of the right foot, obesity, hypertension, and hyperlipidemia.

The clinical importance of this baseline cannot be overstated. Charcot neuroarthropathy is a well-recognized, progressive complication of diabetic peripheral neuropathy involving pathological bone remodeling and joint destruction caused by repetitive, undetected microtrauma in the insensate foot — a process that occurs entirely independent of acute traumatic events. The right foot trajectory documented in these records, including a January 2022 subacute fracture, the November 2023 arthrodesis of the proximal interphalangeal joint, and the 2025 Lisfranc joint complex dislocation, is fully consistent with progressive diabetic Charcot neuroarthropathy advancing along its natural history.

B. Absence of Post-Accident Right Foot Complaints (April 2023 – May 2025)

A review of all Grady Health System encounters between April 28, 2023 and June 2025 reveals no documentation of right foot pain, swelling, instability, or injury attributed to or temporally associated with the April 28, 2023 motor vehicle accident. The records reflect multiple clinical encounters during this interval for the management of Ms. Watkins' chronic comorbidities — including diabetes management, podiatry, nephrology, ophthalmology, wound care, and dermatology — without any complaint of foot injury referable to the accident. This 26-month absence of documented MVA-related foot symptoms following the alleged injury is clinically inconsistent with an acute traumatic Lisfranc joint injury caused by the subject accident.

C. Absence of Lisfranc Diagnosis at All Post-MVA Evaluations Prior to June 2025

Of particular significance is the complete absence of any Lisfranc injury diagnosis, Lisfranc joint complaint, or midfoot instability finding at every clinical evaluation documented in the Grady Health System records between the date of the MVA (April 28, 2023) and the June 2025 pre-operative encounter. The post-MVA encounters that specifically addressed the right foot — all predating any Lisfranc diagnosis — are enumerated below:

June 29, 2023 (DPM, Dr. Santamarina): Right foot first toe ulcer with depth to fat; right hallux malleus; nail damage. X-ray of right foot ordered. Assessment and plan document wound debridement and follow-up. No Lisfranc complaint, no midfoot instability, no Lisfranc diagnosis.

July 26, 2023 (Wound/Ostomy RN): Right first toe wound care follow-up. Patient asked about x-ray results. No midfoot symptoms, no Lisfranc complaint.

August 9, 2023 (DPM, Dr. Santamarina): Right first toe ulcer resolved. Toenail care. Assessment: hallux malleus, diabetic polyneuropathy. No Lisfranc finding.

September 20, 2023 (DPM, Dr. Santamarina): Follow-up for right foot ulcer. Second x-ray of right foot ordered. Assessment and surgical planning for right first proximal interphalangeal joint arthrodesis. No Lisfranc complaint, no midfoot pathology documented.

November 13, 2023 (DPM, Dr. Santamarina): Right first toe ulcer; nonreducible flexure contracture of right first interphalangeal joint. X-ray of right foot ordered. Patient scheduled for surgery on November 30, 2023. Operative indications: right foot ulcer (depth to fat), right hallux malleus, diabetic polyneuropathy. No Lisfranc injury, no Lisfranc diagnosis, no midfoot pathology.

November 30, 2023 (Surgery, Dr. Santamarina): Right first proximal interphalangeal joint (PIPJ) arthrodesis with fixation and application of skin substitute allograft. Pre-operative and post-operative diagnoses: (1) right foot ulcer, depth to fat; (2) right hallux malleus; (3) diabetic polyneuropathy. Intraoperative fluoroscopy confirmed appropriate fusion position at the first IP joint. No Lisfranc injury in any operative diagnosis, no midfoot surgical finding.

December 6 and December 13–15, 2023 (DPM post-operative visits): Status post right first PIPJ arthrodesis. Incision healing, sutures removed, post-operative shoe. No Lisfranc complaint, no midfoot symptom documented.

2024 (multiple encounters): Ongoing management of diabetic gastroparesis, retinopathy, nephrology, and routine podiatric care including toenail management and callus care. No right foot Lisfranc complaint, no midfoot instability, no Lisfranc diagnosis across any 2024 encounter reviewed.

Lisfranc's sprain (right foot) and the associated external cause code W22.09XA first appear in the Grady Health System problem list on July 3–15, 2025 — at or following the ORIF surgery. This represents the first documentation of a Lisfranc diagnosis across the entire medical record, occurring more than twenty-six months after the subject MVA and contemporaneous with — not preceding — the surgical intervention. The Lisfranc injury cannot be causally attributed to the April 28, 2023 motor vehicle accident when it was not identified as a clinical entity at any evaluation during the intervening period, including evaluations by the same treating podiatric surgeon who ultimately performed the Lisfranc ORIF.

D. Lisfranc Joint Pathology: Radiographic and Etiologic Findings

When right foot imaging was obtained in June–July 2025, the radiographic findings demonstrated degenerative changes at the first metatarsotarsal joint and heterotopic ossification surrounding the first tarsometatarsal articulation. The radiologist's report explicitly characterized these findings as potentially related to “end-stage renal disease or Charcot arthropathy” — not to acute trauma. These chronic, degenerative radiographic features are inconsistent with an acute traumatic Lisfranc injury from April 2023 and are entirely consistent with the long-standing Charcot neuroarthropathy process documented throughout this patient's medical record.

E. Surgical Course and Complications

Right foot Lisfranc open reduction and internal fixation was performed on July 3, 2025 and was followed by rapid hardware failure requiring revision surgery on July 17, 2025 fourteen days later. Radiographs on July 15 demonstrated persistent widening of the Lisfranc interval and possible hardware migration, including a K-wire in the superficial plantar soft tissues. Rapid hardware failure of this type is a recognized potential complication of Lisfranc fixation in patients with Charcot neuroarthropathy, where absent protective sensation enables continued undetected loading that overwhelms surgical fixation. Uncontrolled diabetes further impairs osseous healing and dramatically increases susceptibility to fixation failure.

Following revision surgery, Ms. Watkins developed a polymicrobial wound infection involving *Enterobacter cloacae* (identified in the July 3 intraoperative culture but not treated with targeted antibiotics until August 2025), *Klebsiella pneumoniae*, and *Streptococcus agalactiae*, requiring inpatient debridement and prolonged antibiotic therapy. She subsequently developed euglycemic diabetic ketoacidosis (August 26–29, 2025), exacerbated by her gastroparesis, inability to tolerate oral intake, and concurrent SGLT-2 inhibitor use. These downstream complications are directly attributable to her underlying comorbidities — not to an acute traumatic event from April 2023.

The following opinions are expressed to a reasonable degree of medical probability based on the records reviewed:

The core reasoning supporting these opinions rests on four independent pillars:

1. Temporal Disconnect and Absence of Lisfranc Diagnosis at Intervening Evaluations

Clinically significant traumatic Lisfranc injuries — particularly those requiring open surgical reconstruction — produce immediate and recognizable symptoms including acute foot pain, swelling, inability to bear weight, and ecchymosis in the plantar arch. The absence of any such presentation following April 28, 2023 is inconsistent with acute traumatic causation. More critically, across every right foot evaluation documented between April 2023 and June 2025 — including three separate DPM visits with dedicated right foot x-rays, a right foot surgery, and multiple post-operative visits by the same treating podiatric surgeon — no clinician identified, suspected, or recorded a Lisfranc injury, Lisfranc sprain, or midfoot instability. The diagnosis of Lisfranc's sprain first entered the medical record on the day of the 2025 surgery, not contemporaneously with the accident. An injury that goes undiagnosed across all intervening evaluations, including a right foot surgery by the same treating podiatrist, does not appear to have a temporal relationship to the cited incident.

2. Pre-Existing Disease Trajectory

Ms. Watkins' right foot had been on a documented pathological trajectory prior to the April 2023 accident. A January 2022 emergency evaluation documented a right foot subacute fracture in the setting of Charcot neuroarthropathy. Multiple 2022–2023 podiatric visits documented progressive diabetic foot disease including ulceration and joint contracture of the right first toe. The November 2023 arthrodesis addressed a right toe ulcer with rigid interphalangeal joint deformity — pathology consistent with progressive diabetic foot disease independent of the April 2023 accident. The 2025 Lisfranc pathology represents a third orthopedic event in an already structurally compromised right foot — a progression fully explained by the natural history of Charcot neuroarthropathy without requiring a traumatic precipitant from April 2023.

3. Patient-Reported Denial of Trauma

At the June 24, 2025 pre-operative evaluation — the most proximate Grady clinical encounter before the index Lisfranc surgery — Ms. Watkins denied any accidents or trauma as the cause of her right foot symptoms. This self-reported denial, documented by her treating surgical team, directly contradicts a causation theory linking the foot pathology to the April 2023 accident. The subsequent appearance of an impact-mechanism ICD-10 code added at or after surgery does not override the contemporaneous pre-operative denial and does not establish the April 2023 accident as the precipitating event.

4. Biological Plausibility of Charcot Progression

Charcot neuroarthropathy in the setting of diabetic peripheral neuropathy is medically well-established as a progressive, self-perpetuating process. Absent protective sensation means Ms. Watkins cannot sense the repetitive microtrauma that gradually destroys midfoot ligamentous and

bony architecture. This process does not require a single dramatic traumatic event. The Lisfranc joint complex is one of the most commonly affected sites in Charcot foot. In a patient with documented Charcot neuroarthropathy, absent protective sensation, poorly controlled diabetes (HbA1c 9.0–9.8%), and a prior right foot arthrodesis, progressive Lisfranc joint disruption represents the expected natural history — not an accident-attributable injury. The radiographic appearance of degenerative and heterotopic ossification changes at the first tarsometatarsal joint, characterized as chronic in nature and attributed by the radiologist to possible end-stage renal disease or Charcot arthropathy, further supports this conclusion.

This opinion is based exclusively on the medical records produced from Grady Health System and is subject to revision upon receipt of additional records, police reports, deposition transcripts, independent medical examination findings, or other case materials.

These opinions are based on my knowledge and experience regarding diagnostic and clinical care of orthopedic diagnoses during my work in my own clinical practice. As a licensed orthopedic surgeon, I feel qualified to comment on the above orthopedic matters. The opinions rendered in this case are the opinions of the undersigned. This evaluation has been conducted based on the medical record documentation provided with the assumption that the material is true and correct. If more information becomes available, an additional service/report/reconsideration may be requested. Such information may or may not change the opinions rendered in this evaluation. This opinion is based on clinical assessment of the patient's records and documentation only.

A handwritten signature in black ink, appearing to read 'Saman Vojdani', with a stylized, flowing script.

7/1/2026

Saman Vojdani, MD

Board Certified, Orthopedic Surgery ABOS